

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

COMMITTEE SUBSTITUTE
FOR ENGROSSED
HOUSE BILL NO. 4457

By: Newton and Deck of the
House

and

Jech, Coleman, Murdock,
Dossett, Haste, and Stanley
of the Senate

COMMITTEE SUBSTITUTE

An Act relating to pharmacy benefits managers (PBMs);
defining terms; prohibiting PBM from engaging in
certain actions; requiring certain PBM to offer
certain pharmacy certain opportunities; prohibiting
pharmacy network from denying participation to
certain pharmacy; requiring certain provisions to be
applied equally; requiring certain PBM to make
certain written standards available upon request;
requiring certain PBM to approve or deny certain
application within certain time period; allowing
certain denial to be subject to appeal; requiring
certain PBM to permit certain person to obtain
certain medication in certain situations; construing
provisions; requiring certain PBM to maintain certain
records for certain time period; constituting
violation of certain provisions as an unfair or
deceptive act; allowing Attorney General to
promulgate rules as necessary; allowing Attorney
General to engage in certain actions; providing for
codification; and providing an effective date.

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

1 SECTION 1. NEW LAW A new section of law to be codified
2 in the Oklahoma Statutes as Section 6958.1 of Title 36, unless there
3 is created a duplication in numbering, reads as follows:

4 A. As used in this section:

5 1. "Health insurer" means any corporation, association, benefit
6 society, exchange, partnership, or individual licensed by the
7 Oklahoma Insurance Code;

8 2. "Medically integrated pharmacy" means a pharmacy that:

- 9 a. is owned by, affiliated with, or under common
10 ownership with a specialty provider or specialty
11 provider practice, or under contract with a specialty
12 provider or specialty provider practice to provide
13 pharmacy services in connection with the provider's
14 treatment of a covered person,
15 b. is physically or operationally integrated within the
16 clinical practice of the specialty provider,
17 c. is licensed and in good standing with the State Board
18 of Pharmacy, and
19 d. employs at least one pharmacist licensed in this state
20 to dispense prescription medications;

21 3. "Network pharmacy" means pharmacy providers contracted with
22 a pharmacy benefits manager;

23 4. "Specialty medication" means a prescription drug that:
24

- a. is used to treat complex, chronic, or rare medical conditions and requires specialized handling, administration, monitoring, or patient management, or
- b. is classified or designated as a specialty medication by the pharmacy benefits manager or the health benefit plan pursuant to written, clinically based criteria that is applied uniformly and is not for the purpose or effect of steering a covered person to a pharmacy owned by, affiliated with, designated by, or preferred by the pharmacy benefits manager;

5. "Specialty provider" means a licensed physician or other licensed practitioner, acting within his or her scope of licensure, who provides ongoing specialized medical treatment requiring complex medication management including, but not limited to, providers practicing in the fields of oncology, urology, neurology, nephrology, dermatology, cardiology, anesthesiology, or orthopedics;

6. "Patient steering" means any communication, representation, benefit design, network practice, prior authorization requirement, claims edit, reimbursement policy, or administrative act that has the purpose or effect of inducing, requiring, pressuring, or causing a covered person or specialty provider to use a pharmacy owned by, affiliated with, designated by, or preferred by the pharmacy benefits manager instead of a medically integrated pharmacy otherwise qualified pursuant to this section; and

1 7. "Pharmacy benefits manager" means the same as defined in
2 Section 6960 of Title 36 of the Oklahoma Statutes.

3 B. A pharmacy benefits manager (PBM) shall not:

4 1. Restrict or prohibit a covered person from obtaining a
5 specialty medication from a medically integrated pharmacy affiliated
6 with the covered person's treating specialty provider; provided,
7 that such pharmacy is licensed and in good standing with the State
8 Board of Pharmacy and participating in the pharmacy network;

9 2. Require that a covered person obtain a specialty medication
10 exclusively through a pharmacy owned by, affiliated with, or
11 designated by the PBM when the covered person's treating specialty
12 provider maintains a medically integrated pharmacy capable of
13 dispensing such medication and meets the requirements set forth in
14 paragraph 1 of this subsection;

15 3. Discriminate against or disadvantage a medically integrated
16 pharmacy in the terms and conditions of network participation,
17 including by imposing credentialing, inventory, shipping, or
18 reporting requirements, reimbursement terms, fees, or accreditation
19 or audit standards that are more restrictive or less favorable than
20 those applied to similarly situated pharmacies;

21 4. Impose additional or more stringent administrative
22 requirements on a medically integrated pharmacy that are not equally
23 imposed on other network pharmacies dispensing specialty
24 medications;

1 5. Deny, reduce, or delay reimbursement for a specialty
2 medication solely because the medication is dispensed by a medically
3 integrated pharmacy or because the medication is dispensed by a
4 medically integrated pharmacy;

5 6. Require a specialty provider or covered person to transfer a
6 valid prescription to another pharmacy as a condition of coverage or
7 reimbursement;

8 7. Impose differential co-payments, coinsurance, or other cost-
9 sharing on a covered person based solely on the pharmacy from which
10 the specialty medication is obtained, or otherwise impose benefit
11 design or administrative requirements that are intended to steer, or
12 have the effect of steering, a covered person away from a medically
13 integrated pharmacy;

14 8. Engage in patient steering or any practice that has the
15 effect of directing patients away from a medically integrated
16 pharmacy to a pharmacy owned or affiliated with such PBM;

17 9. Implement any policy, prior authorization requirement, step
18 therapy protocol, or utilization management practice that has the
19 effect of: preventing a specialty provider from dispensing
20 medications necessary to ensure continuity of care in accordance
21 with the provisions of Section 6570.57 of Title 36 of the Oklahoma
22 Statutes; reducing unnecessary treatment delays in accordance with
23 the provisions of Sections 6570.50 through 6570.59 of Title 36 of
24 the Oklahoma Statutes; or unreasonably delaying, interfering with,

1 or denying access to clinically appropriate therapy through a
2 medically integrated pharmacy;

3 10. Retaliate against, terminate, subject to retaliatory audit
4 or review, or refuse to contract with or renew the contract of any
5 medically integrated pharmacy, specialty provider, or covered person
6 for asserting any rights established pursuant to this section,
7 filing a complaint, participating in an investigation, or providing
8 information to the Attorney General; or

9 11. Evade or avoid the provisions of this section through an
10 affiliate, subsidiary, agent, vendor, subcontractor, health benefit
11 plan, network administrator, utilization management entity, or any
12 other person or entity acting on behalf of the PBM.

13 C. A pharmacy network shall not restrict or deny participation
14 of a medically integrated pharmacy in such pharmacy network on the
15 basis of accreditation status, provided that such pharmacy maintains
16 current accreditation from at least one of the following accrediting
17 organizations:

- 18 1. URAC;
- 19 2. Accreditation Commission for Health Care (ACHC);
- 20 3. Joint Commission (JC); or
- 21 4. National Association of Boards of Pharmacy (NABP).

22 D. A PBM shall offer a medically integrated pharmacy the same
23 opportunity to participate in specialty pharmacy networks as is
24 offered to any other pharmacy. Any reimbursement rate, fee

1 schedule, audit standard, credentialing requirement, or contractual
2 term applied by a PBM to a medically integrated pharmacy shall be no
3 less favorable than those applied to any specialty pharmacy owned by
4 or affiliated with such PBM. A PBM shall make available, upon
5 written request, the written standards for specialty pharmacy
6 network participation, credentialing criteria, reimbursement
7 methodology, material contract terms, and any accreditation or
8 clinical service requirements applicable to a medically integrated
9 pharmacy. A PBM shall approve or deny a completed application for
10 participation in a specialty pharmacy network within thirty (30)
11 calendar days after receipt of such written request and shall
12 provide written notice stating, with specificity, the basis of any
13 denial of such request. Any denial of such request may be subject
14 to appeal.

15 E. When a specialty provider is actively managing a covered
16 person's treatment plan, including monitoring for adverse effects,
17 drug interactions, or therapeutic outcomes, the PBM shall permit the
18 covered person to obtain specialty medications from the specialty
19 provider's medically integrated pharmacy and to participate in the
20 pharmacy network. Nothing in this subsection shall be construed to
21 require a PBM to cover a drug that is not otherwise covered by the
22 health insurer. Nothing in this subsection shall be construed to
23 prohibit a PBM from applying prior authorization requirements,
24 quantity limits, or other utilization management standards that are

1 based on nationally recognized clinical criteria, are supported by
2 written policies, and are applied uniformly to all similarly
3 situated pharmacies and covered persons without regard to pharmacy
4 affiliation.

5 F. A PBM shall maintain, for not less than five (5) years, all
6 records necessary to demonstrate compliance with this section
7 including, but not limited to, network participation criteria,
8 credentialing records, contracts, reimbursement schedules, policies,
9 procedures, algorithms, communications relating to specialty
10 pharmacy routing, or claims adjudication, prior authorization, or
11 audit records used to direct or steer covered persons to a pharmacy.
12 Such records shall be produced to the Attorney General upon request.

13 G. 1. A violation of this section shall constitute an unfair
14 or deceptive act and shall be subject to enforcement by the Attorney
15 General. The Attorney General may promulgate rules as necessary to
16 implement and enforce the provisions of this section.

17 2. The Attorney General may receive complaints, conduct
18 investigations, issue subpoenas or civil investigative demands,
19 require written responses under oath, examine books and records, and
20 bring an action in district court to enforce the provisions of this
21 section.

22 3. In any action brought pursuant to the provisions of this
23 section, the Attorney General may seek temporary, preliminary, or
24 permanent injunctive or declaratory relief, restitution,

1 disgorgement, civil penalties, attorney fees, costs, or any other
2 relief authorized by law. Each claim, prescription denial, or day
3 of continuing noncompliance may constitute a separate violation.

4 SECTION 2. This act shall become effective November 1, 2026.

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